

On-Call Birth Bag Protocol

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The birth bag remains packed by the front door year-round. Bag contents are verified at the start of every on-call week. This protocol covers bag contents, pre-shift communication, transport, at-birth documentation, and handoff procedures.

Birth Bag Contents Checklist

Clinical - Verify Weekly

Item	Quantity	Verified
Fetoscope and Doppler with charged battery	1 ea	■
Ultrasound gel (travel size)	2 bottles	■
Sterile gloves, size M and L	2 pairs ea	■
Non-sterile exam gloves, box	1 box	■
Birth kit (sterile)	1	■
Cord clamps	4	■
Cord scissors (sterile)	1 pair	■
Bulb syringe	2	■
Sterile gauze pads (4x4)	10 pads	■
IV supplies (cannula 18G and 20G, tubing, LR 500mL)	2 of each	■
Pitocin (oxytocin) 10 units/mL - per protocol	4 vials	■
BP cuff (manual, calibrated)	1	■
Pulse oximeter	1	■
Thermometer with covers	1	■
Portable oxygen (2L tank, mask, cannula)	1 set	■
Infant resuscitation bag-mask (newborn size)	1	■
Newborn hat and blanket (sterile)	2 ea	■
Placenta basin (plastic, large)	1	■

Personal - Verify Start of Shift

- Charged phone and backup battery pack
- Phone charger and car charger
- Protein snacks (bars, nuts) - enough for 12 hours
- Water bottle (32 oz minimum)
- Change of clothes in a separate bag

- Printed route sheet and backup directions (offline capable)
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Pre-Shift Communication

Call chain on first day of every on-call rotation:

- Confirm availability with the backup CNM covering the secondary call slot
- Text collaborating OB's service line with on-call start and end time
- Verify that answering service has the correct on-call number active
- Confirm with partner (Marcus) the call schedule - especially any overnight or weekend calls

If a client calls with active labor signs, obtain: contraction frequency and duration, rupture of membranes status, fetal movement, and whether any transport barriers exist. Dispatch ETA to client and alert the backup hospital labor and delivery unit.

Transport Protocol

To birth location: All planned home births are mapped ahead of time. The route sheet includes the primary address, the nearest cross streets, and GPS coordinates. For clients living in East Lake, Reynoldstown, Kirkwood, or Ormewood Park, direct routes to Grady Memorial and Emory Midtown are pre-loaded.

Emergency transport: If intrapartum transport becomes necessary, call 911 immediately and simultaneously alert the backup OB. Do not attempt to transport alone in non-emergency situations - request a second attendant to drive. Provide ETA and SBAR handoff by phone while en route. Bring the complete birth bag and the client's birth chart.

At-Birth Documentation

The following must be recorded with exact timestamps during and immediately after the birth:

- Time of full dilation confirmation
 - Time of crowning and time of birth (head delivery, body delivery)
 - APGAR scores at 1 minute and 5 minutes
 - Weight, length, and head circumference
 - Time of placenta delivery and placenta inspection findings
 - Estimated blood loss
 - Any resuscitation measures or interventions
 - Newborn initial feeding: time, method, observed latch/intake
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Postpartum Handoff

Before leaving the birth location, complete the following:

- Maternal vitals stable: BP, pulse, fundal tone, lochia character
- Newborn vitals stable: temp, resp, color, tone, feeding initiated

- Written discharge instructions provided and reviewed with client
- Next visit scheduled: 24-hour newborn weight check and 48-hour postpartum visit
- 24-hour call number confirmed with client
- Notify collaborating OB of birth outcomes by secure message within 2 hours
- Complete birth documentation in the clinical record within 4 hours of leaving the birth location